3.4 Chronic Obstructive Pulmonary Disease (COPD)

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I cannot climb stairs without stopping." **Objective** • Barrel chest • Breathes out through pursed lips • SpO_2 88%, $PaCO_2$ 52 mmHg	**ACTUAL** **Impaired gas exchange** related to alveolar destruction and air trapping as evidenced by hypoxemia, hypercapnia and dyspnea.	SpO_2 88–92% (or the prescribed target) within 24 hours.	• Give controlled oxygen to the prescribed target; avoid high concentrations. • Watch ABG, SpO_2 and level of consciousness. • Sit him upright or leaning forward. • Teach pursed-lip and diaphragmatic breathing. • Give bronchodilators and steroids as prescribed.	SpO_2 on target and alert: **goal met**. Report drowsiness (he may be retaining CO_2).
Subjective • "I get tired when I wash or dress." **Objective** • Breathless with very little effort • HR goes up by 25/min with activity	**ACTUAL** **Activity intolerance** related to imbalance between oxygen supply and demand as evidenced by exertional dyspnea and fatigue.	Manages his ADL with rests, keeping SpO_2 on target.	• Plan rest periods between activities. • Teach ways to save energy (sit down to wash, keep things close by). • Encourage pulmonary rehabilitation. • Check SpO_2 while he is active.	Gets through ADL with breathlessness he can tolerate: **goal met**.
Subjective • Smokes, 40 pack-years **Objective** • Secretions thick • On steroids • Admitted often	**RISK** **Risk for infection** as evidenced by risk factors: chronic lung disease, retained secretions and corticosteroid therapy.	No new chest infection.	• Encourage influenza and pneumococcal vaccines. • Teach the early signs of infection (sputum changing colour or amount, fever). • Help him stop smoking. • Encourage fluids and clearing secretions.	No signs of infection: **goal met**.